Arrhythmias & Hypertensive Emergencies – Section Summary

Tachy/brady • WPW • Long QT • Pacemakers • Hypertensive emergencies

Critical Care Board Review • Section 6 • MCQ-Oriented

1. Tachyarrhythmias

- Post-CABG wide-complex → **VT**
- COPD + irregular P waves → **MAT** → treat cause, O_2, **Mg**, ± CCB
- AVNRT → **adenosine**
- Unstable AF/flutter → **DC cardioversion 120 J**
- Stable VT → **amiodarone**

2. WPW & Long QT

- Short PR + delta wave = **pre-excitation (bundle of Kent)**
- WPW + AF → avoid AV-nodal blockers (procainamide/DC)
- Long QT + deafness → **Jervell–Lange-Nielsen** → β-blocker

3. Bradycardia & Pacing

- Wenckebach = **Mobitz I** (AV node)
- AV dissociation, slow escape → **third-degree block**
- Hyperkalaemia → bradycardia/blocks
- Spikes without QRS → **failure to capture** → CXR, electrolytes, ischaemia
- New effusion after pacing → **lead perforation**

4. Hypertensive Emergencies

- Flash pulmonary oedema → **nitroprusside/nitro + diuresis**, MAP ↓ ≤ 25%
- Type B dissection with malperfusion → **TEVAR**
- Cocaine: **benzodiazepine** first
- Asymptomatic severe hypertension → no emergency treatment

5. High-Yield MCQ Pearls

- ☑ MAT → treat cause + Mg
- ☑ WPW + AF: no AV blockers

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